Fund for Innovation in Development (FID) — Call for Proposals 2026

Innovation Proposal

FairBanks Community Health Intelligence Platform (FCHIP)

Predictive community health that keeps families out of poverty

Your health, our mission.

FairBanks Community Reach — Health for All — Obulamu eri Bonna · Afya kwa Wote · Oburamu bwa Boona

Aligned with the FID Call for Proposals 2026. Source

Item	Detail
Applicant	FairBanks Medical Centre / FairBanks Community Reach (social enterprise, Uganda)
Sectors	Health · Technology & Innovation · Economic Inclusion
Geography	Uganda (pilot) → East Africa → broader ODA-eligible markets
Funding stage	Requested stage: Pilot Grant (up to €200,000) — pathway to Impact Evaluation & Scale-Up

1. Executive Summary

In underserved African communities, illness is not only a health problem — it is one of the fastest routes into poverty. When an outbreak, a high-risk pregnancy, or an untreated chronic condition is caught late, families face catastrophic out-of-pocket costs, lost income, and deepening inequality. Today most primary healthcare is reactive: facilities treat patients only after they arrive sick, while the data that could warn everyone earlier sits scattered across community health workers' notebooks, outreach registers, and siloed systems.

FairBanks Community Health Intelligence Platform (FCHIP) is a deep-technology innovation that turns community-generated health data into early warnings and decision support. Community Health Workers (CHWs) and Village Health Teams (VHTs) capture structured data on offline-capable mobile tools; the platform applies artificial intelligence, machine learning, and GIS mapping to predict disease outbreaks, maternal complications, chronic-disease hotspots, child-health threats, and medicine stock-outs — then routes actionable alerts to CHWs, clinics, districts, insurers, and partners before crises escalate.

FairBanks is uniquely positioned to test this innovation. We already operate a functioning medical centre and active community outreach across Kampala-area communities (Bukoto, Kyebando, Kisaasi, Kamwokya, and Kikaaya), working with CHWs and VHTs on maternal and child health, geriatric care (Gericare), and chronic-disease screening. This gives us a live, real-world environment to pilot, evaluate, and refine FCHIP — exactly the evidence-first pathway FID is built to fund.

We are applying for a Pilot Grant to validate FCHIP under real-world conditions and generate the early evidence needed to progress toward a rigorous impact evaluation and, ultimately, scale-up across ODA-eligible markets.

2. The Problem: When Illness Deepens Poverty

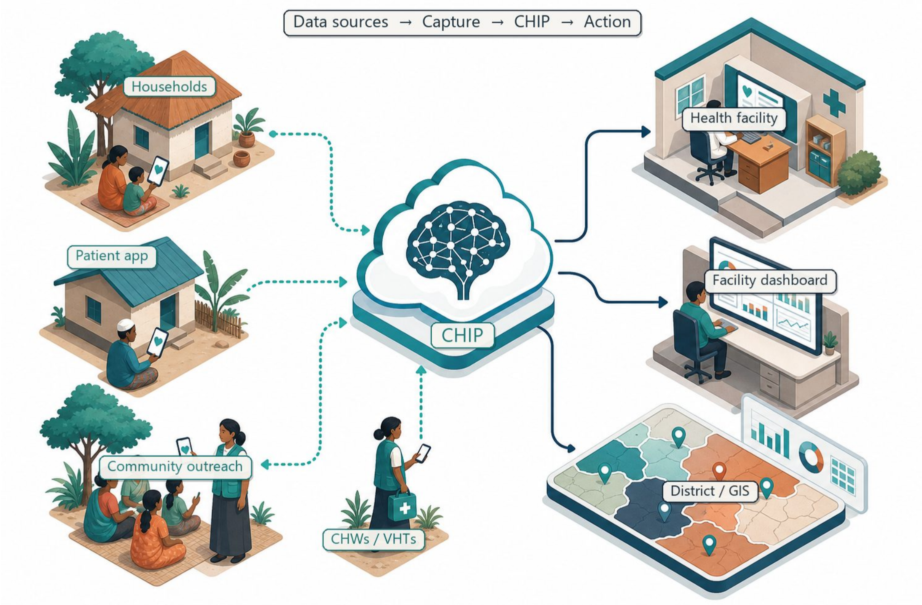
Reactive care waits for sickness before it responds

Across underserved African communities, a structural information gap keeps health systems reactive — with a direct poverty and inequality cost.

Current reality	Consequence
Care starts only after people fall sick	Late detection of outbreaks and complications; higher treatment costs
Community data sits in paper registers and silos	Districts, NGOs, and insurers lack real-time intelligence
High-risk pregnancies and NCDs found late	Preventable maternal deaths and stroke/diabetes burden
Medicine ordered on guesswork	Stock-outs during seasonal disease surges
Health shocks hit poor households hardest	Catastrophic spending pushes families deeper into poverty

Existing digital tools rarely combine multi-source community data, predictive modelling, geospatial mapping, and real-time decision support — leaving fragmented data and missed prevention.

3. The Innovation: FCHIP



Community signals → capture → FCHIP intelligence → action

FCHIP turns community-generated health data into predictions and decision support, sitting between last-mile capture and the people who can act — CHWs, clinics, districts, insurers, and partners.

3.1 Deep technology core

Technology	Function in FCHIP
Artificial Intelligence	Outbreak early warning; maternal and NCD risk scoring
Machine Learning	Pattern learning from community data, seasonal trends, outreach outcomes
GIS Mapping	Disease distribution, hotspots, and resource-gap visualisation
Mobile Data Collection	Offline-capable CHW/VHT capture at household level
Cloud Computing	Secure sync across facilities, partners, administrative levels
Analytics & NLP	Live dashboards; local-language symptom capture where useful

3.2 Predictive use cases

Disease surveillance: Rising fever reports across neighbouring villages → predicted malaria surge → targeted testing, bed-nets, and pharmacy pre-stocking.

Maternal health: Home-visit BP, anaemia proxies, and ANC adherence → high-risk pregnancy flags → early CHW alerts and referral.

Non-communicable diseases: Community and workplace BP/glucose screening → hypertension and diabetes hotspots → targeted campaigns.

Child health: Growth, immunisation, and diarrhoea data → malnutrition and coverage-gap alerts → nutrition and immunisation drives.

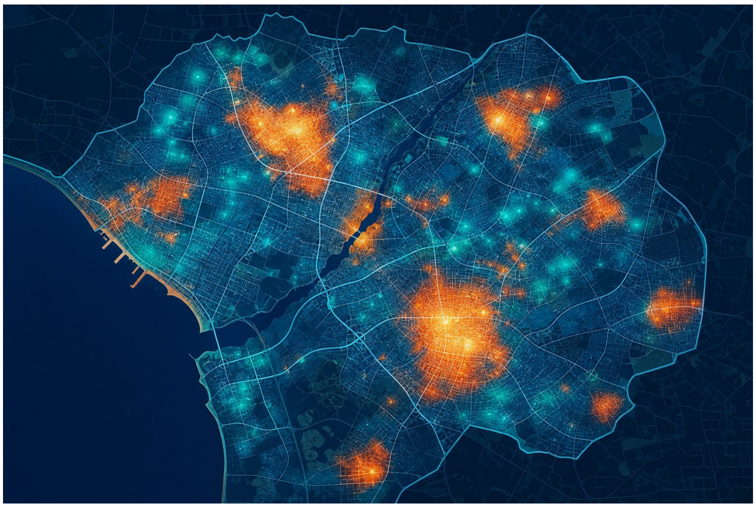
Medicine demand: Disease trends, rainfall, and consumption → demand forecasts → replenishment before stock-outs.

4. Theory of Change

A clear, testable theory of change links this health innovation directly to poverty and inequality reduction:

Stage	Detail
Inputs	FID Pilot Grant; FairBanks live operations; CHW/VHT networks; mobile + cloud + AI stack.
Activities	Build offline capture app, cloud pipeline, AI risk models, GIS layer, and dashboards; train CHWs; run pilot.
Outputs	Structured community health data; predictive alerts; facility/district dashboards; referrals triggered.
Outcomes	Earlier detection and referral; fewer missed high-risk cases; fewer stock-outs; better-targeted outreach.
Impact	Reduced preventable illness and health-driven poverty; lower inequality for underserved communities.

5. Evidence of Impact & Evaluation



GIS hotspot mapping — measurable, testable outputs

- Existing evidence: CHW/mHealth programmes and community-based early warning have documented gains in maternal, child, and communicable-disease outcomes across sub-Saharan Africa; FCHIP builds on this base by adding predictive intelligence across multiple data sources.
- Baseline data: FairBanks outreach screening, maternal programmes, pharmacy dispensing, and facility encounters already generate the signals needed to establish a credible pre-intervention baseline.
- Measurable outcomes: alert accuracy, time-to-referral, proportion of high-risk pregnancies identified early, immunisation and screening coverage, and stock-out frequency.
- Evaluation pathway: the pilot is designed so that a subsequent counterfactual impact evaluation (comparison communities) can rigorously test effect sizes — the qualifying step toward FID's Impact Evaluation and Scale-Up stages.

6. Cost & Cost-Effectiveness

- Builds on assets that already exist — a live medical centre, trained CHW/VHT networks, and outreach programmes — so the pilot avoids the cost of standing up field operations from scratch.
- Offline-first, mobile-based capture keeps hardware and connectivity costs low and works in low-bandwidth settings.
- Prevention is cheaper than late treatment: early detection of outbreaks, maternal risk, and NCDs reduces costly emergency care and lost livelihoods.
- A shared platform serves many users (CHWs, clinics, districts, insurers), spreading cost and improving cost per beneficiary as coverage grows.

7. Scale & Financial Sustainability

A shared intelligence layer that scales across users and districts

- Technical scalability: cloud architecture and standard mobile tools extend from one catchment to many districts without redesign.
- Model portability: CHW/VHT systems exist across Uganda and much of Africa, so the approach transfers to other ODA-eligible markets.
- Financial sustainability: diversified revenue — clinic/hospital subscriptions, district and Ministry of Health deployments, NGO programme monitoring, insurer analytics, and partner APIs — reduces reliance on grants over time.
- Policy pathway: evidence generated in the pilot supports integration into public health planning and district decision-making, aligning with FID's public-policy funding streams.

8. Why FairBanks — Traction & Foundation

Live FairBanks Community Reach outreach in Kampala communities

- Functioning FairBanks Medical Centre with direct patient and community access
- Active Community Reach outreach in Bukoto, Kyebando, Kisaasi, Kamwokya, Kikaaya, and surrounding areas

- Established CHW and VHT relationships and trust in target communities
- Live programmes: maternal & child health, Gericare, chronic-disease screening, corporate & school health
- Digital health records foundation and pharmacy operations already generating data signals
- Research and community partnerships that support ethical data use and evaluation

9. Implementation Plan & Budget

9.1 Roadmap

Phase	Timeline	Milestones
Phase 1 — Pilot	0–12 mo	MVP live with FairBanks CHWs/VHTs; validate 3 use cases; establish baseline
Phase 2 — Impact evaluation prep	12–18 mo	Counterfactual design; comparison communities; measurement framework
Phase 3 — District scale	18–36 mo	Partner clinics and district structures; NGO/insurer modules; APIs
Phase 4 — Scale-up	Year 3+	Multi-district Uganda; East Africa entry; policy integration

9.2 Indicative pilot budget

Budget line	Detail	Amount
Product & engineering	Mobile capture app, cloud pipeline, AI/GIS modules, dashboards	€70,000
Field pilot & CHW training	CHW/VHT onboarding, devices, supervision, community engagement	€45,000
Data, evaluation & M&E;	Baseline, monitoring, evaluation design toward counterfactual study	€35,000
Data governance & compliance	Consent, anonymisation, Uganda Data Protection alignment	€15,000
Project management & operations	Coordination, reporting, partnerships, contingency	€35,000
Total (indicative Pilot Grant)		€200,000

10. Alignment with FID & the SDGs

10.1 FID evaluation criteria

FID criterion	How FCHIP responds
Evidence of impact	Clear theory of change, baseline data, and a pathway to counterfactual evaluation
Cost & cost-effectiveness	Builds on existing assets; prevention reduces costly late treatment
Scale & sustainability	Portable model; diversified revenue; policy-integration pathway
Poverty & inequality focus	Targets underserved communities where health shocks drive poverty

10.2 Sustainable Development Goals

SDG	FCHIP contribution
SDG 1 — No Poverty	Prevents health-shock-driven catastrophic spending and lost income
SDG 3 — Good Health & Well-Being	Predictive community health; maternal/child/NCD focus
SDG 5 — Gender Equality	Women-led venture; maternal health intelligence
SDG 10 — Reduced Inequalities	Intelligence and care for underserved communities

SDG	FCHIP contribution
SDG 17 — Partnerships	Government, NGO, insurer, and academic collaboration

11. Risks & Mitigation

Risk	Mitigation
Data quality from field capture	Structured forms, validation rules, CHW training and supervision
Low CHW digital literacy	Simple offline UX, local-language support, supervised rollout via VHTs
Privacy and consent	Ethical governance, anonymisation, Uganda Data Protection compliance
Early model accuracy	Start rule-based + limited ML; validate against FairBanks clinical outcomes
Public-sector adoption	Pilot evidence; district co-design; alignment with MoH community health strategy

12. Conclusion

From community signals to life-saving, poverty-reducing predictions

FCHIP fits the FID model precisely: a promising innovation, ready to be tested in a real-world setting, with a clear pathway from pilot to rigorous evaluation and scale. By catching health risks before they become crises, FairBanks helps underserved families avoid the catastrophic costs that trap them in poverty — a genuine win for communities and for development.

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